

The impact of judges' perceptions of credibility in fibromyalgia claims.

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Fibromyalgia (FM) is a confusing and controversial diagnosis, characterized by widespread pain and tenderness at specific anatomical sites. The cause of this syndrome is unknown, and the course of the condition is difficult to predict. Without a known cause, predictable course, or effective treatment, it is not surprising that FM is a contentious diagnosis from a medical perspective, as well as a civil litigation and disability insurance industry perspective. The purpose of this study was to investigate judges' perceptions of credibility in litigated cases involving FM claims in the Canadian courts, and the relation between perceived credibility and awards granted. A systematic review was conducted of every trial-by-judge litigated FM claim in Canada (N=194 cases) up to 2003. The cases were examined in relation to credibility factors. The role and responsibility of the plaintiff was central in claims involving issues of misrepresentation, fraud, non-disclosure, failure to mitigate, and contributory negligence. The presence of these issues suggested a possible decrease or loss in the claim as a result of the plaintiff's conduct. In regards to the actions of defendants, the presence of investigative and surveillance information alone did not affect the awards granted. However, the credibility of that information had a large effect on the amount of award granted. Plaintiff credibility played a similar role, indicating that plaintiffs perceived as more credible were typically granted greater awards. An examination of medical expert credibility revealed that judges appear to perceive experts as more credible overall than plaintiffs, regardless of the expert's role in the case.

Becoming familiar with fibromyalgia.

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Although the term "fibromyalgia" has appeared in the literature for 90 years, the disease may go unrecognized by clinicians. It is a devastating disease, more common than rheumatoid arthritis, yet its cause remains a mystery. There is no cure for fibromyalgia; however, nurses may be able to help improve the quality of life for the patients who live with this disease. This article explores theories of causation, treatment modalities, and the potential role of nurses in their work with these patients.

Non-inflammatory pain in inflammatory arthritis.

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'Non-inflammatory' pain, pain that is not associated with measures of inflammation, is common in patients with inflammatory arthritis including RA. One important cause of non-inflammatory pain is concomitant fibromyalgia. Systematic review has shown that fibromyalgia is common in inflammatory arthritis including RA affecting 1 in 5 patients and is associated with higher disease activity scores due to inflated tender joint count and patient global assessment. Consequently, many patients with RA and concomitant fibromyalgia may fail to reach treatment target and switch to alternate disease modifying drugs frequently. European Alliance of Association for Rheumatology has highlighted that concomitant fibromyalgia is an important consideration in assessing difficult-to-treat RA. The incidence and prevalence of fibromyalgia are higher in RA than the general population, raising the possibility that fibromyalgia may be 'secondary' to RA rather than a concomitant disease. The precise mechanisms

whereby patients with RA develop fibromyalgia are unknown. In this review, we discussed fibromyalgia in RA, its clinical impact and epidemiology as well as data suggesting fibromyalgia might be 'secondary'. Lastly, we reviewed potential pathogenic mechanisms which included inflammatory cytokines sensitizing nociceptive neurones, temporal summation, also known as windup, from chronic pain and impaired coping from poor quality sleep and mental well-being. Deciphering the exact mechanisms may lead to treatment strategies that prevent development of secondary fibromyalgia and will address a common factor associated with difficult-to-treat RA.

Fibromyalgia.

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Fibromyalgia is characterized by chronic, widespread musculoskeletal pain and associated fatigue, sleep disturbances, and other cognitive and somatic symptoms. For many patients, these symptoms persist for years and lead to frequent health care use; for some, fibromyalgia and its symptoms can be debilitating. Although many treatments are available, management remains challenging. This article highlights the clinical features of fibromyalgia, discusses diagnostic criteria and their evolution, and reviews treatment options.

Environmental factors and fibromyalgia syndrome: a narrative review.

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This in-depth review of fibromyalgia (FM), which is a complex condition characterised by chronic pain, fatigue, sleep disturbances, and a spectrum of diagnostically and therapeutically challenging symptoms, underlines the need for a comprehensive and integrated approach that also takes into account the psychological factors affecting patient responses. We focus on the substantial impact that environmental factors (climatic variations, air pollution, electromagnetic field exposure, physical and emotional traumas, dietary patterns, and infections) have on the manifestation and intensity of symptoms, and advocate personalised, holistic treatment of patients' psychological and environmental sensitivities by suggesting the benefits of tailored dietary and stress management. We also call for further research into the complex interplay of environmental, biological and psychological factors influencing FM in order to develop more effective individualised treatments that are capable of enhancing patient care and outcomes.

Schwann cell TRPA1 elicits reserpine-induced fibromyalgia pain in mice.

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Fibromyalgia is a complex clinical disorder with an unknown aetiology, characterized by generalized pain and co-morbid symptoms such as anxiety and depression. An imbalance of oxidants and antioxidants is proposed to play a pivotal role in the pathogenesis of fibromyalgia symptoms. However, the precise mechanisms by which oxidative stress contributes to fibromyalgia-induced pain remain unclear. The transient receptor potential ankyrin 1 (TRPA1) channel, known as both a pain sensor and an oxidative

stress sensor, has been implicated in various painful conditions. The feed-forward mechanism that implicates reactive oxygen species (ROS) driven by TRPA1 was investigated in a reserpine-induced fibromyalgia model in C57BL/6J mice employing pharmacological interventions and genetic approaches. Reserpine-treated mice developed pain-like behaviours (mechanical/cold hypersensitivity) and early anxiety-depressive-like disorders, accompanied by increased levels of oxidative stress markers in the sciatic nerve tissues. These effects were not observed upon pharmacological blockade or global genetic deletion of the TRPA1 channel and macrophage depletion. Furthermore, we demonstrated that selective silencing of TRPA1 in Schwann cells reduced reserpine-induced neuroinflammation (NADPH oxidase 1-dependent ROS generation and macrophage increase in the sciatic nerve) and attenuated fibromyalgia-like behaviours. Activated Schwann cells expressing TRPA1 promote an intracellular pathway culminating in the release of ROS and recruitment of macrophages in the mouse sciatic nerve. These cellular and molecular events sustain mechanical and cold hypersensitivity in the reserpine-evoked fibromyalgia model. Targeting TRPA1 channels on Schwann cells could offer a novel therapeutic approach for managing fibromyalgia-related behaviours.

B-cell and plasma cell activation in a mouse model of chronic muscle pain.

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Fibromyalgia (FM) is a complex chronic musculoskeletal pain disorder with an elusive pathogenesis, with a strong implication of immune interactions. We recently found that IL-5 and the adaptive immune system mediates pain outcomes in fibromyalgia (FM) patients and preclinical models of FM-like chronic widespread pain (CWP). However, there is an active debate if FM/CWP has an autoimmune etiology. Preclinical models of CWP utilize a repeated insult paradigm, which resembles a primary, then secondary response similarly observed in the antibody response, in which the subsequent event causes a potentiated pain response. Recent translational studies have implicated immunoglobulins (Ig) and B-cells in FM/CWP pathophysiology. To understand if these are involved in preclinical models of CWP, we performed comprehensive B-cell phenotyping in the bone marrow, circulation, and popliteal (draining) lymph nodes in the two-hit acidic saline model of CWP. We found increased MHC class II-expressing B-cells in peripheral blood, increased activated plasma cells in peripheral blood, and increased memory B-cells in the bone marrow. Interestingly, acidic pH (4.0) injected mice have reduced levels of IgG1, independent of treatment with IL-5. We have demonstrated that the acidic saline model of CWP induces T-cell mediated activation of B-cells, increased active plasma cells, and increased memory B-cells in female mice.

Fibromyalgia: Pathogenesis, Mechanisms, Diagnosis and Treatment Options Update.

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Fibromyalgia is a syndrome characterized by chronic and widespread musculoskeletal pain, often accompanied by other symptoms, such as fatigue, intestinal disorders and alterations in sleep and mood. It is estimated that two to eight percent of the world population is affected by fibromyalgia. From a medical point of view, this pathology still presents inexplicable aspects. It is known that fibromyalgia is caused by a central sensitization phenomenon characterized by the dysfunction of neuro-circuits, which involves the perception, transmission and processing of afferent nociceptive stimuli, with the prevalent manifestation of pain at the level of the locomotor system. In recent years, the pathogenesis of

fibromyalgia has also been linked to other factors, such as inflammatory, immune, endocrine, genetic and psychosocial factors. A rheumatologist typically makes a diagnosis of fibromyalgia when the patient describes a history of pain spreading in all quadrants of the body for at least three months and when pain is caused by digital pressure in at least 11 out of 18 allodermal points, called tender points. Fibromyalgia does not involve organic damage, and several diagnostic approaches have been developed in recent years, including the analysis of genetic, epigenetic and serological biomarkers. Symptoms often begin after physical or emotional trauma, but in many cases, there appears to be no obvious trigger. Women are more prone to developing the disease than men. Unfortunately, the conventional medical therapies that target this pathology produce limited benefits. They remain largely pharmacological in nature and tend to treat the symptomatic aspects of various disorders reported by the patient. The statistics, however, highlight the fact that 90% of people with fibromyalgia also turn to complementary medicine to manage their symptoms.

Fibromyalgia severity and symptoms are associated with the disorders of gut-brain interaction.

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Fibromyalgia remains an idiopathic common disorder characterised by widespread pain with no universally accepted treatment. Irritable bowel syndrome is prevalent among women living with fibromyalgia. The prevalence of other disorders of gut-brain interaction (DGBI) and associations with fibromyalgia symptoms and severity is unknown. To evaluate the prevalence of the range of DGBI and associations with the symptoms and severity of fibromyalgia in women. A prospective observational study was conducted in New Zealand in 2022. A comprehensive survey included validated measures to identify DGBI (Rome IV) and items assessing the severity of fibromyalgia and pain symptoms, sleep quality, quality of life, mental health and migraine. Analysis was conducted employing Spearman's rho, Mann-Whitney U, Kruskal-Wallis and chi-square tests. A total of 111 adult women with fibromyalgia enrolled in the study. Of these, 98 (93%) met the criteria for at least one DGBI, and 67 (68%) satisfied criteria for more than one. All groups of DGBI, and 11 specific DGBI were significantly associated with measures of pain, fibromyalgia severity, sleep problems and migraine ($p < 0.05$). Severity of pain and symptoms associated with fibromyalgia, including sleep problems, were also significantly associated with the functional bowel disorder severity index. This study demonstrated that the prevalence of DGBI in women with fibromyalgia extends beyond irritable bowel syndrome. Presence of multiple DGBI correlates with pain, severity indices of fibromyalgia and sleep problems. Further research is required to examine the aetiology of DGBI in this population. This observational study has identified important relationships between the broader DGBI, fibromyalgia pain and associated symptoms, particularly migraine and sleep disturbance. Notable correlations between the severity indices of each are demonstrated, suggesting that improvements in one domain may reduce pain and improve overall well-being. These findings highlight the importance of addressing each clinical feature of the condition when supporting patients with fibromyalgia.

[Fibromyalgia syndrome--pathogenesis, diagnosis, and treatment problems].

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Fibromyalgia is a fairly common syndrome characterized by chronic, widespread musculoskeletal pain, multiple "tender points", fatigue, sleep disturbance, stiffness and other symptoms such as headache, dizziness, trouble with concentration, irritable bowel syndrome, urinary urgency, depression. Fibromyalgia may occur at any age, even in childhood, is much more common in women than in men. The cause of fibromyalgia is unknown. It is difficult to diagnose because many of the symptoms are similar to symptoms of other disorders. There are no laboratory tests that can confirm a diagnosis of fibromyalgia. Average time from onset to diagnosis is 5-8 years. Treatment of fibromyalgia requires a comprehensive approach.
